

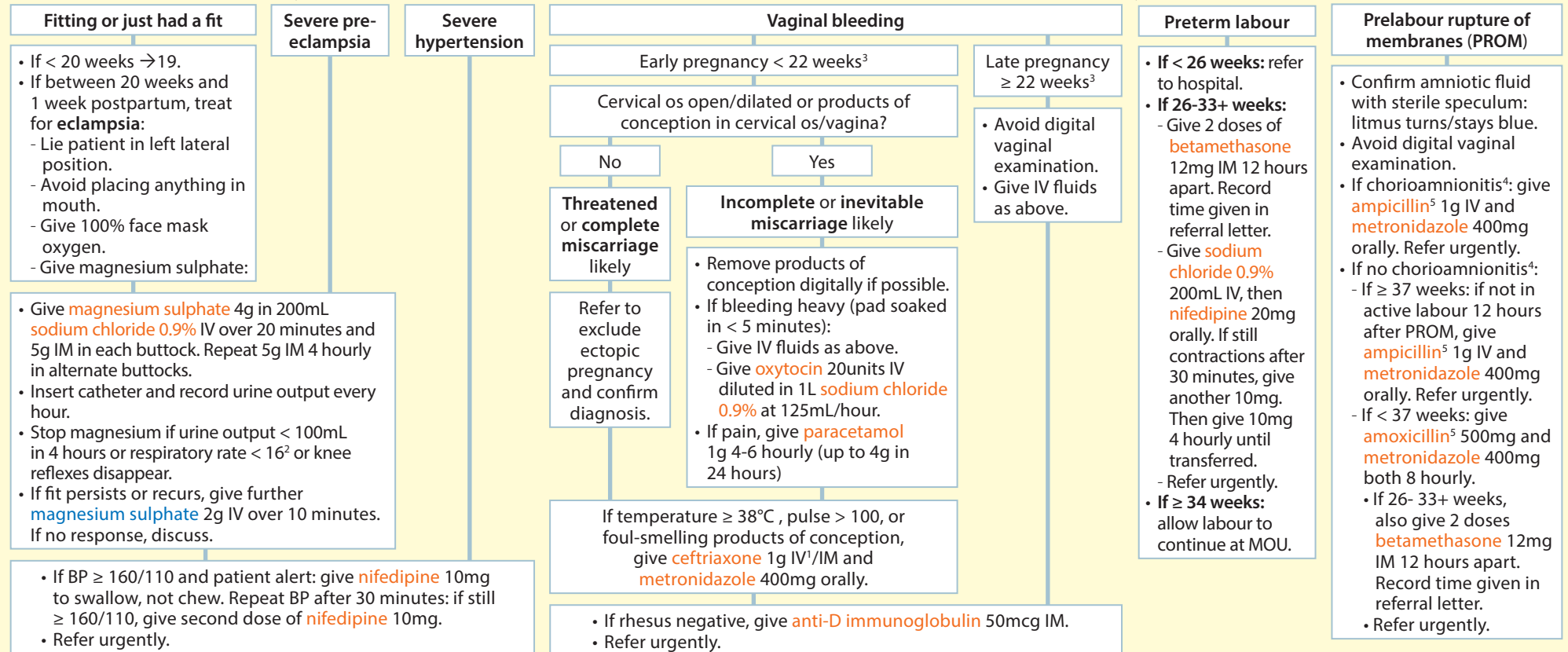
ROUTINE ANTENATAL CARE

Give urgent attention to the pregnant patient with any of:

- Fitting or just had a fit
- BP $\geq 140/90$ and persistent headache/blurred vision/abdominal pain: treat as **severe pre-eclampsia**
- BP $\geq 160/110$ and $\geq 1+$ proteinuria: treat as **severe pre-eclampsia**
- BP $\geq 160/110$ without proteinuria: treat as **severe hypertension**
- Painful contractions < 37 weeks: **preterm labour** likely
- Sudden gush of clear or pale fluid from vagina with no contractions: **prelabour rupture of membranes (PROM)** likely
- Vaginal bleeding
- Temperature $\geq 38^\circ\text{C}$ and severe back or abdominal pain
- Difficulty breathing
- Swollen painful calf
- Decreased/no fetal movements $\hookrightarrow$ 161.

Manage urgently:

- If difficulty breathing, give face mask oxygen and refer urgently.
- If BP < 90/60, give **sodium chloride 0.9%** 500mL IV over 30 minutes, repeat until systolic BP > 90. Continue 1L 6 hourly. Stop if breathing worsens. Refer urgently.
- If temperature $\geq 38^\circ\text{C}$ and difficulty breathing/back pain/abdominal pain, give **ceftriaxone** 1g IV¹/IM unless PROM (see below). Refer urgently.



¹Do not mix Ringer's lactate and IV ceftriaxone. Flush IV line with **sodium chloride 0.9%** before and after IV ceftriaxone. ²If respiratory rate < 16, give **calcium gluconate** 10% 10mL IV slowly over 2 minutes. ³If gestation not known, manage as late pregnancy if uterus palpable above umbilicus. ⁴Temperature $\geq 38^\circ\text{C}$, painful abdomen or foul-smelling amniotic fluid. ⁵If penicillin allergy (history of anaphylaxis, urticaria or angioedema), give instead **azithromycin** 500mg daily.